

ST. MARY'S MEDICAL CENTER

Comprehensive Health Assessment

Patient: Michael Thompson

Date: November 5, 2024

Age / Gender: 58 years / Male

Physician: Dr. Sarah Johnson

VITAL SIGNS

Blood Pressure	148/92 mmHg (resting)	Heart Rate	78 bpm
Respiratory Rate	16 breaths/min	Temperature	36.7°C
Oxygen Saturation	97%	BMI	29.8 (Overweight)

LABORATORY RESULTS

Test	Result	Reference Range	Status
Glucose (fasting)	142 mg/dL	70–99 mg/dL	ELEVATED
Hemoglobin A1c	7.2%	< 5.7%	ELEVATED
Total Cholesterol	228 mg/dL	< 200 mg/dL	ELEVATED
LDL Cholesterol	155 mg/dL	< 100 mg/dL	HIGH
HDL Cholesterol	42 mg/dL	> 40 mg/dL (M)	LOW
Triglycerides	185 mg/dL	< 150 mg/dL	ELEVATED
Creatinine	1.1 mg/dL	0.7–1.2 mg/dL	Normal
eGFR	72 mL/min	> 60 mL/min	Normal
AST	32 U/L	10–40 U/L	Normal
ALT	28 U/L	7–56 U/L	Normal

PHYSICAL EXAMINATION

Cardiovascular	Regular rhythm, no murmurs. Slightly elevated JVP.
Respiratory	Clear to auscultation bilaterally. No wheezes or crackles.
Abdomen	Soft, non-tender. No organomegaly.
Musculoskeletal	Full range of motion all joints. No edema.
Neurological	Grossly intact. Cranial nerves II–XII intact.

PATIENT HISTORY

Symptoms	Occasional headaches, fatigue, and mild shortness of breath when climbing stairs. No chest pain, syncope, or palpitations.
Family History	Father: myocardial infarction at age 62. Mother: type 2 diabetes.
Social History	Former smoker (quit 5 years ago, 20 pack-year history). Occasional alcohol use (2–3 drinks/week).
Current Medications	Metformin 500mg twice daily (6 months) · Atorvastatin 20mg once daily (3 months) · Lisinopril 10mg once daily (1 month)

DIAGNOSES

1. Type 2 diabetes mellitus, poorly controlled (HbA1c 7.2%)
2. Hyperlipidemia, mixed
3. Stage 1 hypertension
4. Obesity (BMI 29.8)

RECOMMENDATIONS

1. Increase Metformin to 1000mg twice daily.
2. Consider adding Amlodipine 5mg daily if blood pressure remains elevated.
3. Increase Atorvastatin to 40mg daily.
4. Referral to ophthalmology for diabetic eye exam.
5. Referral to podiatry for diabetic foot exam.
6. Dietician consultation for diabetic and heart-healthy diet planning.
7. Exercise program: 30 minutes of moderate activity, 5 days per week.
8. Follow-up in 3 months with repeat HbA1c, lipid panel, and renal function tests.

CLINICAL NOTES

Patient expressed motivation to improve diet and exercise. Advised to monitor blood glucose at home. Diabetes education materials provided. Smoking cessation reinforced. Immunizations are up to date — flu shot administered today. Colonoscopy screening due next year.